

Stakeholder Analysis and Engagement Plan

HealthFirst Care – Patient Experience & Operational Efficiency Initiative

This document identifies the stakeholders involved in HealthFirst Care's patient-experience initiative, classifies them by influence and interest, and defines how each group should be engaged. It draws directly on the named individuals in the Stakeholder Profiles document and the objectives defined in the Business Requirements Document (BRD) from the prior labs.

Task 1: Stakeholder Identification & Categorization

Six stakeholder groups are relevant to this initiative. Five (Patients, Doctors, Nurses, Administrative Staff, IT Teams) are detailed with named individuals in the Stakeholder Profiles document. "Hospital Leadership / Project Sponsor" is named in the assignment scenario as a stakeholder type but has no individual profile in the source materials; it is included here as the body that mandates and funds the project, consistent with the BRD's reference to sponsor-level objectives.

Stakeholder Group	Representative Individuals	Role / Responsibility	Influence Level	Interest Level
Patients	Sarah Ayvazyan (35, Software Engineer); Lak Ayer (62, Retired Teacher)	Receive care; book and attend appointments; give feedback on service quality.	Low	High
Doctors	Dr. Aftab Khan (Cardiology); Dr. Robert Lee (Orthopaedics)	Deliver clinical care; depend on scheduling, equipment access, and timely inter-department information.	High	High
Nurses	Santa Murmu (Paediatric Nurse); Jessica Gomes (Emergency Room Nurse)	Coordinate day-to-day patient care, staffing, and department handoffs.	Low	High
Administrative Staff	Maria Carter (Appointment Scheduler); Ivan Walker (Billing Administrator)	Manage scheduling, records, and billing operations.	Low	High
IT Teams	Rajesh Singh (IT Manager); Laura Simkow (Software Developer)	Own system architecture, integration, security, and technical delivery of the solution.	High	High
Hospital Leadership / Project Sponsor	Not individually profiled in source materials	Approves budget and scope; accountable for project outcomes and organizational priorities.	High	High

Task 2: Stakeholder Influence/Interest Matrix

Influence is defined as a stakeholder's power to affect project decisions, budget, or adoption; interest is how directly the project outcome affects them day to day. Placements below reflect this project team's analysis of the case materials, not a stakeholder-validated survey.

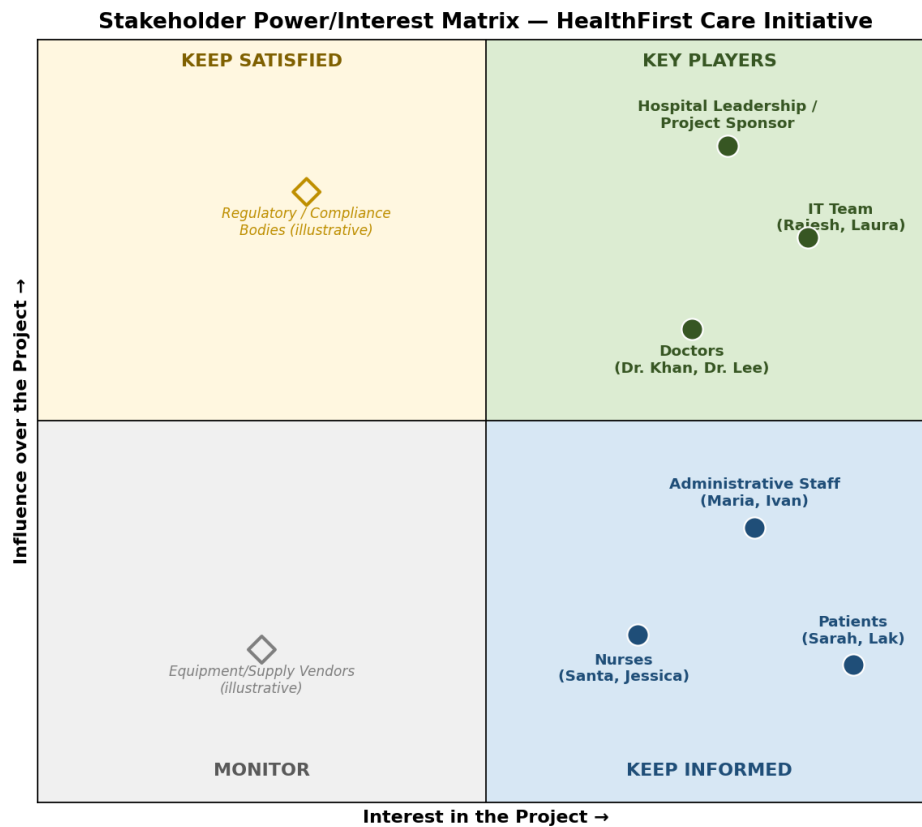


Figure 1 — Stakeholder power/interest matrix. Diamond markers are illustrative examples of stakeholder types not detailed in the provided case materials, included only to show what would typically occupy the remaining two quadrants.

Quadrant	Stakeholder Group(s)	Rationale
Key Players (High Influence, High Interest)	Hospital Leadership / Project Sponsor; IT Teams; Doctors	These groups both shape and are directly affected by the project: leadership controls funding/scope, IT owns technical delivery, and doctors' adoption determines clinical success.
Keep Satisfied (High Influence, Low Interest)	None identified in the case materials. Illustrative example: regulatory/compliance bodies, who can block or delay the project (high influence) but are not engaged in day-to-day use (low interest).	No profiled stakeholder in the BRD or Stakeholder Profiles document fits this quadrant; it is shown for completeness only.
Keep Informed (Low Influence, High Interest)	Patients; Nurses; Administrative Staff	These groups feel the day-to-day impact of the project (wait times, workflow, scheduling) but do not individually hold authority over project decisions.
Monitor (Low Influence, Low Interest)	None identified in the case materials. Illustrative example: equipment/supply vendors, who have limited stake in this specific initiative.	No profiled stakeholder in the source materials fits this quadrant; it is shown for completeness only.

Task 3: Stakeholder Engagement Plan

Engagement strategy, method, and frequency are tailored to each group's quadrant from Task 2: Key Players receive the deepest, most frequent engagement; Keep Informed groups receive regular one-way and light two-way communication; the illustrative Keep Satisfied/Monitor groups receive periodic, lower-touch updates.

Stakeholder Group	Engagement Strategy	Method(s)	Frequency	Purpose
Hospital Leadership / Project Sponsor	Manage closely — involve in key decisions and secure ongoing sponsorship.	Steering committee meetings; status/decision memos	Monthly, plus ad hoc for major decisions or budget items	Approve scope/budget changes; remove organizational blockers; confirm the project stays aligned with strategic goals.
IT Teams	Manage closely — embed in design and delivery decisions.	Working sessions; sprint/stand-up meetings; shared technical documentation	Weekly (working sessions); daily during active build phases	Validate technical feasibility, integration approach, security, and delivery timeline.
Doctors	Manage closely — consult on workflow-affecting decisions before they are finalized.	Focused interviews; clinical advisory meetings; pilot walkthroughs	Bi-weekly during design; at each major milestone	Confirm scheduling, resource-visibility, and handoff features fit real clinical workflows before rollout.
Nurses	Keep informed — invite input on workflow details without requiring them in every decision meeting.	Short surveys; department briefings; feedback forms	Monthly briefings; survey at each major release	Surface staffing/handoff pain points and confirm proposed workflows are practical on the floor.
Administrative Staff	Keep informed — involve directly in scheduling/records workflow design since they operate the system daily.	Working sessions; user-acceptance testing (UAT) sessions; email updates	Bi-weekly during design/build; every release for UAT	Ensure the scheduling and records features solve the double-booking and record-lookup problems they experience.
Patients	Keep informed — communicate changes and gather feedback without requiring active participation in project governance.	Patient satisfaction surveys; notices/FAQs via email/SMS/portal; suggestion box	Survey after go-live and at each major update; notices as changes occur	Confirm the new scheduling/notification experience addresses wait-time and communication complaints; track satisfaction trend.

Task 4: Summary

This document consolidates the stakeholder identification and classification (Task 1), the power/interest matrix (Task 2), and the tailored engagement plan (Task 3) into a single reference for the HealthFirst Care initiative.

- Key Players (Leadership, IT, Doctors) receive frequent, high-touch engagement because their decisions and buy-in most directly determine whether the project succeeds.
- Keep-Informed groups (Patients, Nurses, Administrative Staff) receive regular, lighter-touch communication and are consulted on workflow details that affect them directly, even though they don't hold formal project authority.

- No stakeholder in the provided case materials fell into the Keep Satisfied or Monitor quadrants; illustrative examples are noted only to show the framework is complete, and should be replaced with real stakeholders (e.g., regulators, vendors) if they are identified as the project progresses.

This plan should be revisited once Hospital Leadership formally confirms sponsorship arrangements and once any regulatory or vendor stakeholders are identified, since neither was detailed in the source documents reviewed for this assignment.